

The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

Data: Our World in Data | IHME Global Burden of Disease (2025) | World, 2023

Hundreds of Millions Affected

1 in 3 Women and 1 in 5 Men Face Major Depression in Their Lifetime

1 in 3

WOMEN

will experience major depression in their lifetime

1 in 5

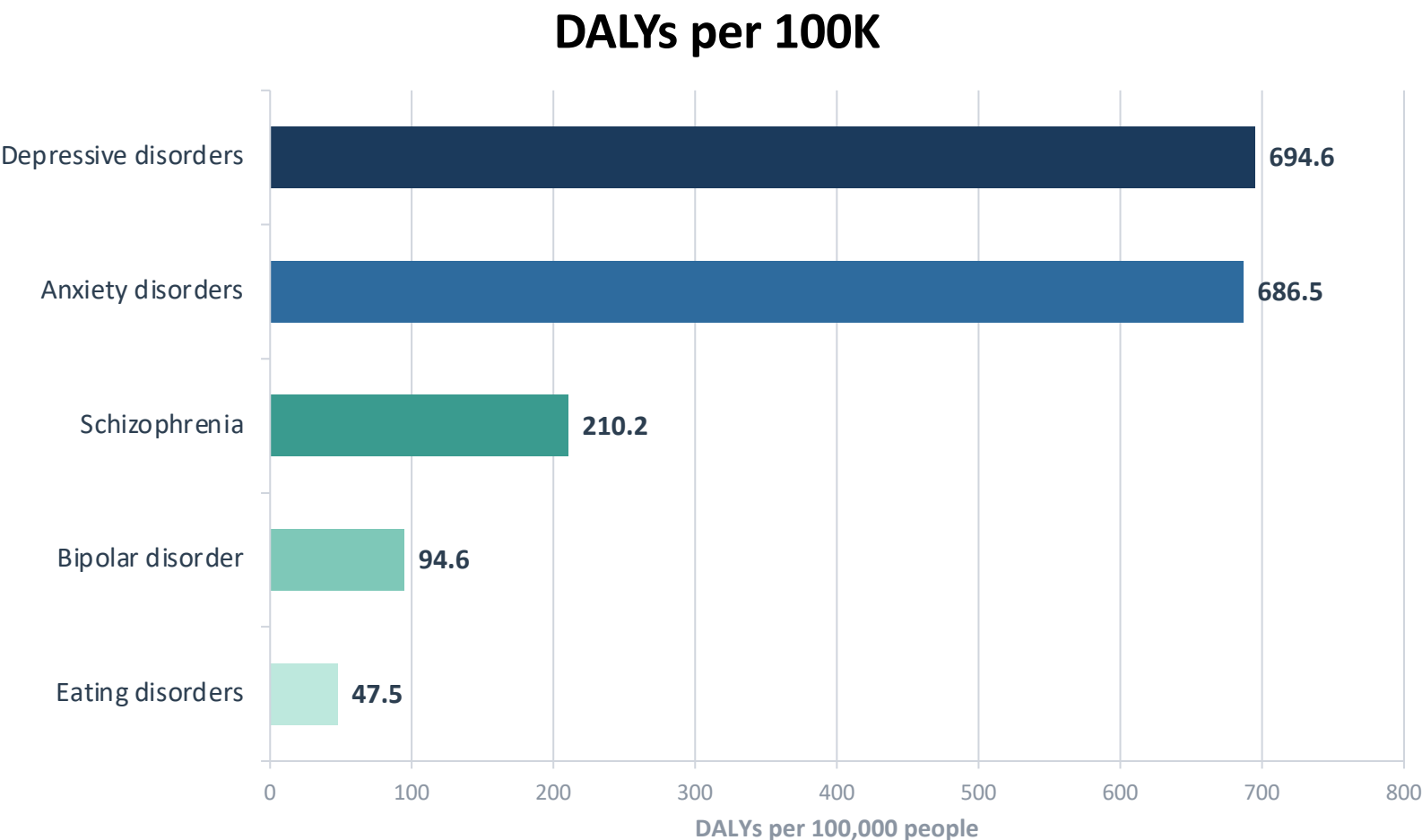
MEN

will experience major depression in their lifetime

Mental health conditions span from common disorders such as depression and anxiety to less common but severe conditions like schizophrenia and bipolar disorder. Hundreds of millions of people suffer from mental health conditions each year, and many more experience them over their lifetimes. Poor mental health affects well-being, the ability to work, and relationships with friends, family, and community.

Depression and Anxiety Dominate the Global Disease Burden

DALYs per 100,000 people by mental illness category, World, 2023



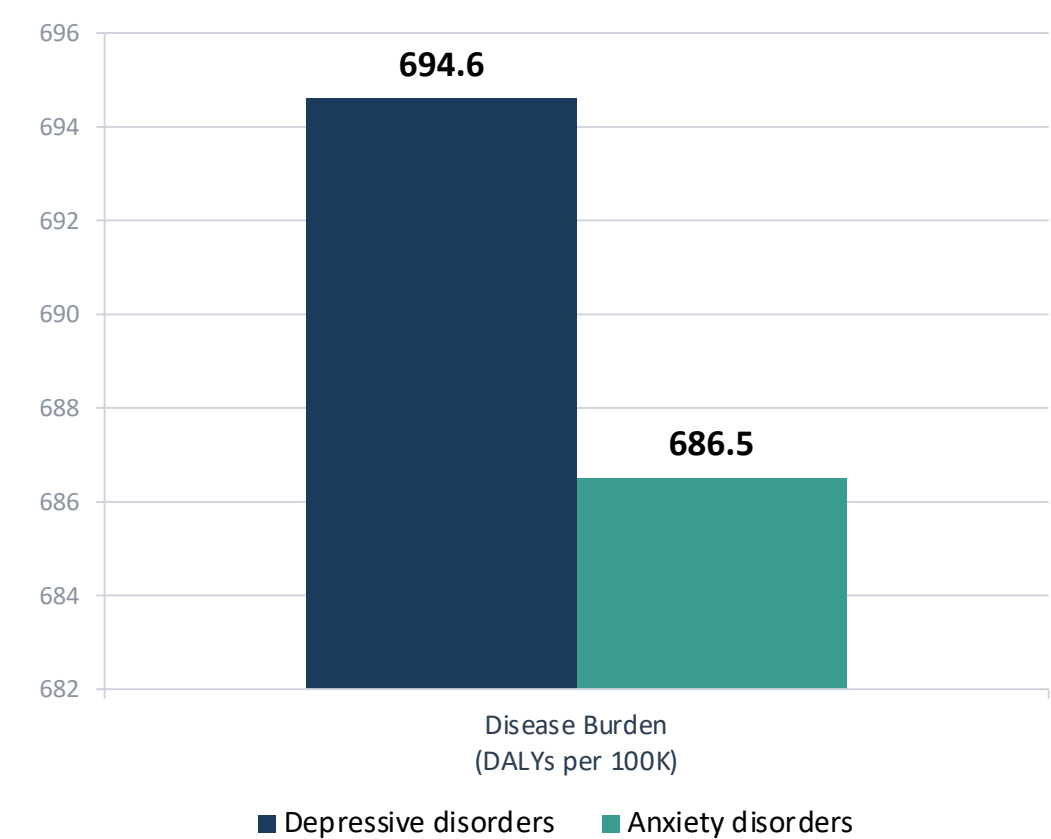
Key Insight

Depressive and anxiety disorders together account for roughly 80% of the total mental health disease burden across these five categories.

~695 and ~687 DALYs per 100K respectively — nearly equal in magnitude.

Depression and Anxiety Carry Nearly Equal Disease Burden Worldwide

Paired comparison of DALYs per 100,000 people, World, 2023



694.6
DALYs per 100K — Depressive Disorders
1 in 3 women / 1 in 5 men lifetime risk

686.5
DALYs per 100K — Anxiety Disorders
The most common mental health condition globally

Difference of only 8.1 DALYs per 100K — near parity

Source: IHME Global Burden of Disease (2025), via Our World in Data

Schizophrenia and Bipolar Disorder: Lower Prevalence but Severe Individual Impact

Proportional comparison of DALYs per 100,000 across all five categories, World, 2023



Depressive disorders 694.6 (40%) Anxiety disorders 686.5 (40%) Schizophrenia 210.2 (12%) Bipolar disorder 94.6 (5%) Eating disorders 47.5 (3%)

Schizophrenia

210.2 DALYs per 100K

Affects fewer people but carries a disproportionately high individual burden, reflecting the severe disability associated with the condition.

Bipolar Disorder

94.6 DALYs per 100K

Lower population-level burden, but individual severity and disability are high. Requires specialised, ongoing care.

Treatment Is Available but Often Lacking or Poor Quality

Barriers that widen the gap between need and care



Stigma

Many people are uncomfortable disclosing symptoms to healthcare professionals or even to people they know.



Access

Treatment is often lacking, especially in low- and middle-income countries where mental health infrastructure is limited.



Quality

Even where treatment exists, it may be of poor quality, reducing effectiveness and patient trust.



Underreporting

Discomfort with disclosure makes it difficult to estimate the true prevalence, meaning the actual burden is likely higher than reported.

Good data is essential to understand these conditions — how, when, and why they occur, and how they can be treated effectively.

How People Cope: From Medication to Faith to Family Conversations

Coping strategies for anxiety or depression vary widely across countries and cultures



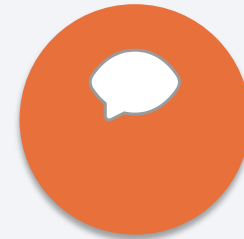
Prescribed Medication

Common in higher-income settings where healthcare infrastructure and access to pharmaceuticals are more readily available.



Religious & Spiritual Activities

Prevalent in many regions worldwide. Faith-based coping plays a significant role in communities where formal mental health services are scarce.



Talking to Friends & Family

A widely reported coping mechanism across cultures. Social support networks are a first line of response for many individuals.

The diversity of coping strategies underscores that mental health responses are shaped by cultural, economic, and healthcare system factors. Policy interventions must account for these differences rather than applying a one-size-fits-all approach.

Key Takeaways: Prioritizing Depression, Anxiety, and Closing the Treatment Gap

1

Depression and anxiety together represent the overwhelming majority of mental health DALYs, each generating approximately 690 DALYs per 100,000 people.

2

Schizophrenia and bipolar disorder carry a disproportionately high individual burden relative to their prevalence, requiring specialised care pathways.

3

Effective treatments exist, but access and quality remain inadequate — particularly in low- and middle-income countries.

4

Stigma suppresses both help-seeking and accurate data, meaning the true burden is likely higher than reported figures suggest.

5

Policy should prioritise scaling evidence-based interventions for common mental disorders while maintaining specialised care for severe conditions.